

Democratic and Popular Algerian Republic

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Course: Medical Deontology

Course Outline:

- I. Introduction
- II. Origins of medical deontology
- III. The code of medical deontology
- IV. Sanctions provided in the code of deontology
- V. The great deontological principles: freedom of the contracting parties
 - The patient's free choice
 - Informed consent
 - The physician's freedom
 - The physician-patient relationship (caregiver-care recipient)
- VI. Conclusion
- VII. Bibliography

I. Introduction:

The word 'deontology' is of Greek origin: 'Deontos' means 'what must be done,' and 'Logos' means 'discourse.'

Deontology is the set of principles, rules and practices (the set of duties and professional rules) that every physician must apply during professional practice. Their observance is general and absolute. In case of non-compliance with one or more rules, the physician incurs ordinal liability (a liability that is a source of sanction).

The rules of medical deontology apply to every physician (or medical student) authorized to practice the profession under the conditions provided by current legislation and regulations.

The rights of patients in the physician/patient relationship are increasingly reinforced, and social

developments — particularly regarding the protection of personal dignity, end-of-life care, biomedical research, and patient safety — are integrated into deontological rules (the duties of physicians toward patients).

There is a code of medical deontology that specifies:

- * The duties of the physician toward fellow physicians.
- * The relations and duties of the physician toward members of other health professions.
- * The duties of the physician toward patients and society.

II. Origins of Medical Deontology:

1) Worldwide:

It has ancient roots — 500 years before Jesus Christ, the Hippocratic Oath had codified medical morality. In 1948, the most current oath was adopted by the World Medical Association in Geneva.

2) In Algeria:

The evolution of medical deontology in Algeria went through several stages:

- Before 1962: the French code of deontology was applicable to every physician authorized to practice in Algeria.
- From 1963: creation of the Office for Supervision of Medical Professions.
- February 1985: promulgation of Law 85/05 on the protection and promotion of health, repealing the code of medical deontology.
- July 1990: promulgation of Law 90-17 amending and supplementing Law 85/05 of 16/02/1985, with Articles 9, 267/1 and 267/2 creating the National Council of Medical Deontology with its 3 national ordinal sections.
- April 1998: official installation of the National Council of Medical Deontology at the Palais de la Culture following national elections.

III. The Code of Medical Deontology:

The foundations of medical deontology are codified in a decree text organized coherently and classified into titles and chapters: the Code of Medical Deontology.

It forms part of the official regulations; it is a legal text whose provisions apply to every physician registered in the medical register, who must swear under oath to respect it. Any breach of its provisions falls under the disciplinary authority of the Medical Council.

The code of deontology reaffirms the traditional principles of medical practice — rules that have been valid since the Hippocratic Oath. It affirms patients' rights and the physician's vocation: dedication, respect for patients' dignity, and all the principles that constitute the duties of the physician.

The code of medical deontology is drafted by a representative authority of the medical

professions: the Medical Council, which is responsible for drafting the deontological rules and submitting them to the Council of State.

It appeared in Executive Decree No. 276 of 06/07/1992 and contains 226 articles distributed across 5 titles:

- General duties of medical deontology
- Professional secrecy
- Duties toward patients
- Duties of collegiality
- Relations of physicians with each other and with members of other health professions
- Special rules for certain modes of practice
- Miscellaneous provisions

IV. Sanctions Provided in the Code of Deontology:

The council must rule within 4 months of receiving a complaint.

Disciplinary sanctions are:

- Warning
- Reprimand
- Proposal to prohibit practice
- Closure of the establishment

=> Sanctions are subject to appeal or review before the National Council of Medical Deontology within 6 months.

=> If unsatisfied, an appeal may again be filed before the Supreme Court within 1 year.

V. The Great Deontological Principles: Freedom of Contracting Parties:

1. The Patient's Free Choice:

The physician must respect the right of every patient to freely choose their physician and must facilitate the exercise of this right.

The physician owes the person they examine, treat or advise loyal, approximate, clear and appropriate information about their condition, the investigations and the care proposed.

Regarding complex situations, such as refusal of care, this is a patient's right that may in certain cases conflict with the physician's duty of assistance when the patient refuses treatment obstinately and unreasonably.

2. Informed Consent:

The consent of the person examined or treated must be sought in all cases. When the patient is able to express their will and refuses the proposed treatment, the physician must respect this

refusal after informing the patient of its consequences.

If the patient is unable to express their will, the physician may not intervene without the family or a close person being notified and informed, except in emergency situations or when it is impossible.

3. The Physician's Freedom:

The physician's independence is one of the foundations of deontology. It is an essential element of the physician/patient relationship in the singular colloquy (the particular relationship).

Within the limits fixed by law and in light of current scientific knowledge, the physician is free in their prescriptions, which should be those they deem most appropriate in the circumstances.

They must limit their prescriptions and actions to what is necessary for the quality, safety and effectiveness of care.

4. The Physician-Patient Relationship (Caregiver-Care Recipient):

This caregiver-care recipient relationship is an integral part of the care contract (the medical contract).

The physician-patient relationship is based on the principle of respect for dignity, the quality of information provided, and the establishment of an adequate therapeutic plan consistent with current scientific knowledge and accepted by the patient. Patient cooperation must play an essential role.

This care relationship evolves over time and adapts to the evolution of the illness and the patient's wishes regarding their health condition.

VI. Conclusion:

The patient, correctly and honestly informed, must constantly express their will through free and unambiguous consent. The patient has the absolute right to be respected and treated with dignity during their passage through the healthcare system. It is their right to receive care that respects their fundamental rights.